EMERGENCY SURGERY · MIDTERM · COMPLETE QUESTION BANK

Emergency Surgery — Midterm All Lectures Question Bank

جراحة / طب الطوارئ — منهج الميدترم — مجمّع أسئلة المادة كلها

4	11	198	59
Lectures	Past exam questions	Expected Qs	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	Expected	Cards
1	Acute Abdomen البطن الحادة	1	59	15
2	Acute Pancreatitis التهاب البنكرياس الحاد	4	39	14
3	Burn الحروق	2	50	15
4	Trauma الإصابات (Trauma)	4	50	15

المصادر:

كويز سابق (جراحة طوارئ)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). الأسئلة المتوقعة عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 4

Trauma

الإصابات (Trauma)

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 An injury caused by the collision of an object with the body in which the object enters the body is called: كويز سابق (جراحة طوارئ)

a. Closed injury
b. Penetrating trauma
c. Internal injury
d. Blunt trauma

4.2 Which of the following is the primary survey in trauma care? كويز سابق (جراحة طوارئ)

a. Head-to-toe assessment
b. (ABCDE) assessment
c. History taking
d. Administering pain relief

4.3 A patient with blunt trauma to the chest develops hypotension and jugular vein distention. What condition should the nurse suspect? كويز سابق (جراحة طوارئ)

a. Hemothorax
b. Pulmonary embolism
c. Pneumothorax
d. Cardiac tamponade

4.4 Which intervention is most appropriate for a patient with a tension pneumothorax? كويز سابق (جراحة طوارئ)

a. Needle decompression
b. Applying a chest binder
c. Nasal oxygen therapy
d. Mechanical ventilation

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

4.N1 Trauma is defined as damage to the body caused by: MCQ

a. Tumors
b. Exchange of energy beyond the body's resilience
c. Infection
d. Degeneration

4.N2 Trauma is the leading cause of death in ages: MCQ

a. Neonates
b. 1–44
c. 45–65
d. Over 65

4.N3 Trauma is the _____ leading cause of death for all ages: MCQ

a. 1st
b. 2nd
c. 3rd
d. 5th

4.N4 **In trauma assessment, the key principle is:** MCQ

a. Take full history first
b. Do CT first
c. Don't get distracted with ugly injuries; initial assessment determines how to proceed
d. Treat the ugliest injury first

4.N5 **Mortality in trauma is decreased when:** MCQ

a. CT is done for every patient
b. The right patient gets to the right hospital in the right time
c. Patients are transported without assessment
d. All patients go to the nearest clinic

4.N6 **Seat belt injuries include all EXCEPT:** MCQ

a. Small intestine injury
b. Ankle fracture
c. Clavicle fracture
d. Mesenteric injury

4.N7 **In acceleration injury, when the car stops suddenly:** MCQ

a. There is no injury
b. Only the legs are injured
c. The person continues moving forwards and the head strikes the car
d. The person stops with it

4.N8 **Firearm injuries by rifles are:** MCQ

a. High velocity injuries
b. Thermal injuries
c. Low velocity injuries
d. Blunt injuries

4.N9 **Kinetic energy formula is:** MCQ

a. $K = M/V$
b. $K = M + V$
c. $K = MV/2g$ ($MV^2/2g$)
d. $K = 2gMV$

4.N10 **In high velocity injuries, shattered fragments of bone act as:** MCQ

a. Secondary missiles
b. Primary missiles
c. Shock waves
d. Blunt trauma

4.N11 **A fracture of the clavicle is an example of:** MCQ

a. Major trauma
b. Penetrating trauma
c. Multiple trauma
d. Minor trauma

4.N12 **Traumatic amputation of a leg is an example of:** MCQ

a. Minor trauma
b. Multiple trauma
c. Major trauma
d. Thermal trauma

4.N13 Multiple trauma is most often the result of: MCQ

a. Stab wounds
b. Falls
c. Motor vehicle accidents
d. Burns

4.N14 The Revised Trauma Score uses all of the following EXCEPT: MCQ

a. Respiratory rate
b. Heart rate
c. Systolic blood pressure
d. Glasgow Coma Scale

4.N15 Each parameter of the Revised Trauma Score is coded from: MCQ

a. 0–2
b. 0–4
c. 1–5
d. 0–10

4.N16 An RTS of less than _____ indicates the need for transport to a trauma center: MCQ

a. 8
b. 9
c. 11
d. 12

4.N17 The triage system sorts patients by: MCQ

a. Age
b. Arrival time
c. Colored labels according to injury and resources availability
d. Sex

4.N18 In the triage system of the lecture, the green label means the patient: MCQ

a. Will die anyway
b. Survives only with medical attention
c. Needs immediate surgery
d. Will survive anyway

4.N19 In the triage system of the lecture, the yellow label means the patient: MCQ

a. Is dead
b. Will survive anyway
c. Will survive only if receiving medical attention
d. Will die anyway

4.N20 ATLS is a protocol for management of: MCQ

a. Burns
b. Cardiac arrest
c. Major trauma
d. Poisoning

4.N21 The correct order of the primary survey is: MCQ

a. ABCDE
b. CABDE
c. DCBAE
d. AMPLE

4.N22 In the primary survey, 'A' stands for: MCQ

a. Analgesia
b. Airway with cervical spine protection
c. Assessment
d. AMPLE history

4.N23 The first question to assess airway patency is: MCQ

a. Did you eat?
b. Is there a fracture?
c. What is your name / can the patient talk?
d. What is the BP?

4.N24 Airway interventions include all EXCEPT: MCQ

a. Chin lift/jaw thrust
b. Suction
c. Needle decompression
d. Oral or nasal airways

4.N25 Cervical spine injury should be suspected in all EXCEPT: MCQ

a. Injury above the clavicle
b. Disturbed conscious level
c. Isolated finger injury
d. Multisystem trauma

4.N26 Cervical spine is immobilized by: MCQ

a. Sandbags only
b. Rigid collar and back board or manual in-line immobilization
c. Neck flexion
d. Soft pillow

4.N27 Airway patency ensures adequate ventilation. TRUE / FALSE

a. True
b. False

4.N28 Popcorn chest refers to: MCQ

a. Flail chest
b. Crepitus (surgical emphysema)
c. Tracheal deviation
d. Sucking wound

4.N29 Tension pneumothorax is treated immediately by: MCQ

a. Needle decompression
b. Intubation
c. Occlusive dressing
d. Chest X-ray first

4.N30 A sucking chest wound is treated by: MCQ

a. Occlusive dressing
b. Leaving it open
c. Needle decompression
d. Chest tube only

4.N31 Hemothorax is treated by: MCQ

a. Chest tube
b. Oxygen only
c. Occlusive dressing
d. Needle decompression

4.N32 Rapid assessment of hemodynamic status includes all EXCEPT: MCQ

a. Pulses
b. Level of consciousness
c. Skin color
d. Blood sugar

4.N33 Shock should be considered in every trauma patient. TRUE / FALSE

a. True **b.** False

4.N34 Cardiac tamponade causes _____ shock: MCQ

a. Hypovolemic **b.** Obstructive
c. Distributive **d.** Cardiogenic

4.N35 Distributive shock is due to: MCQ

a. Disruption of vasomotor tone **b.** Loss of blood
c. Pump failure **d.** Obstruction

4.N36 Circulation management in trauma includes all EXCEPT: MCQ

a. 1 litre Ringer's lactate or blood **b.** Oral fluids
c. Two peripheral IV lines **d.** Direct pressure on bleeding

4.N37 Target urine output in trauma resuscitation is: MCQ

a. 0.5–1 mL/kg/hour **b.** 10 mL/hour
c. No need to monitor **d.** 2–3 mL/kg/hour

4.N38 AVPU is used in: MCQ

a. Circulation **b.** Exposure
c. Airway assessment **d.** Disability (brief neurological exam)

4.N39 In AVPU, 'P' means: MCQ

a. Pressure **b.** Response to painful stimuli
c. Pulse **d.** Pupils

4.N40 Common causes of decreased conscious level in trauma include all EXCEPT: MCQ

a. Hypoxia **b.** Head injury
c. Hypertension **d.** Shock

4.N41 Decreasing mental status may be a sign of elevated intracranial pressure, managed by: MCQ

a. Trendelenburg
b. Fluids restriction only
c. Sitting patient up and hyperventilation
d. Lying flat

4.N42 In exposure, warm blankets are used to prevent: MCQ

a. Pain **b.** Infection
c. Hypothermia **d.** Shock

4.N43 AMPLE history includes all EXCEPT: MCQ

a. Medications **b.** Pulse rate
c. Last meal **d.** Allergies

4.N44 Rectal examination is part of the secondary survey. TRUE / FALSE

a. True **b.** False

4.N45 FAST is: MCQ

a. X-ray **b.** CT scan
c. Blood test **d.** Focused ultrasound for trauma

4.N46 Initial X-rays in trauma include all EXCEPT: MCQ

a. Chest **b.** Pelvis
c. Skull **d.** Cervical spine

4.N47 Hemothorax with more than _____ of blood initially indicates going to the OR: MCQ

a. 500 cc **b.** 1000 cc
c. 1500 cc **d.** 3000 cc

4.N48 Unstable patients with blunt or penetrating abdominal trauma should go to: MCQ

a. Discharge **b.** The OR
c. CT scan first **d.** Ward observation

4.N49 Stable patients with minor or no injuries are: MCQ

a. Admitted **b.** Taken to OR
c. Sent to ICU **d.** Discharged

4.N50 Once a life-threatening injury is discovered, intervention should be delayed until the secondary survey is complete. TRUE / FALSE

a. True **b.** False

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Trauma	Damage from energy exchange beyond the body's resilience with local and systemic effects.	**Epidemiology**	Leading cause of death age 1–44; 3rd for all ages.
Trauma system	Right patient, right hospital, right time.	**Blunt vs penetrating**	Blunt: no skin penetration (RTA, falls). Penetrating: object enters body (stab, gunshot).
Kinetic energy	KE = $MV^2/2g$ — velocity matters most (high-velocity firearms).	**RTS**	GCS + SBP + RR, each 0–4, total 0–12.
ATLS	Primary survey + resuscitation → secondary survey → definitive care.	**A**	Airway with C-spine protection: jaw thrust, suction, airway adjuncts, intubation.

B	100% O_2; needle decompression (tension pneumothorax); chest tube; occlusive dressing.
C	Direct pressure, two large-bore IVs, bloods, 1 L Ringer's or blood; UOP 0.5–1 mL/kg/h.
Shock types	Hypovolemic, cardiogenic, obstructive (tamponade, tension PTX), distributive.
D	AVPU (GCS later); causes: head injury, hypoxia, shock, drugs.
E	Undress fully, inspect back, warm blankets; Foley, Ryle tube, ECG, ABG.
Secondary survey	AMPLE history, head-to-toe exam incl. rectal, reassessment, X-rays, labs, FAST, CT.
To OR	Unstable chest/abdominal trauma, hemothorax > 1500 mL initially, surgical injury on imaging.

3.N44. b — No need for aseptic isolation
Exposure method requires isolation in a completely aseptic atmosphere.

3.N45. c — Cover large raw areas produced by burns
Full-thickness grafts for deep burns of face and hands.

3.N46. b — False
Partial-thickness heal in 2–3 weeks; full-thickness need skin grafts.

3.N47. d — Extent
Mortality ~50% if burn extent is 50%.

3.N48. d — Face
Associated inhalation injury.

3.N49. b — Septicemia

3.N50. a — High-voltage electric burn
Scalds with boiling water are the least harmful.

4.1. b — Penetrating trauma
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.2. b — (ABCDE) assessment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.3. d — Cardiac tamponade
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.4. a — Needle decompression
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.N1. b — Exchange of energy beyond the body's resilience

4.N2. b — 1–44
3rd leading cause for all ages.

4.N3. c — 3rd

4.N4. c — Don't get distracted with ugly injuries; initial assessment determines how to proceed

4.N5. b — The right patient gets to the right hospital in the right time

4.N6. b — Ankle fracture

4.N7. c — The person continues moving forwards and the head strikes the car

4.N8. a — High velocity injuries
Knives and pistols are low velocity.

4.N9. c — K = MV/2g ($MV^2/2g$)
Velocity has the largest effect.

4.N10. a — Secondary missiles

4.N11. d — Minor trauma
Minor: single part/system, e.g. small 2nd-degree burn, cut requiring stitches.

4.N12. c — Major trauma
Serious single-system injury.

4.N13. c — Motor vehicle accidents

4.N14. b — Heart rate

4.N15. b — 0–4
Total RTS 0–12.

4.N16. c — 11

4.N17. c — Colored labels according to injury and resources availability

4.N18. d — Will survive anyway
Lecture: Red = die anyway, Yellow = survive only with medical attention, Green = survive anyway.

4.N19. c — Will survive only if receiving medical attention

4.N20. c — Major trauma
Primary survey + resuscitation → secondary survey → definitive treatment.

4.N21. a — ABCDE

4.N22. b — Airway with cervical spine protection

4.N23. c — What is your name / can the patient talk?

4.N24. c — Needle decompression
Needle decompression is a BREATHING intervention.

4.N25. c — Isolated finger injury
Also maxillofacial trauma.

4.N26. b — Rigid collar and back board or manual in-line immobilization

4.N27. b — False
Airway patency does NOT ensure adequate ventilation.

4.N28. b — Crepitus (surgical emphysema)

4.N29. a — Needle decompression

4.N30. a — Occlusive dressing

4.N31. a — Chest tube

4.N32. d — Blood sugar

4.N33. a — True

4.N34. b — Obstructive
Obstructive: physical obstruction reduces cardiac output.

4.N35. a — Disruption of vasomotor tone

4.N36. b — Oral fluids

4.N37. a — 0.5–1 mL/kg/hour

4.N38. d — Disability (brief neurological exam)
GCS is performed during the secondary survey.

4.N39. b — Response to painful stimuli

4.N40. c — Hypertension
Also alcohol or drug abuse.

4.N41. c — Sitting patient up and hyperventilation

4.N42. c — Hypothermia
Complete disrobing and inspect back.

4.N43. b — Pulse rate
Allergies, Medications, Past history, Last meal, Events.

4.N44. a — True
Physical exam head to toe including rectal exam.

4.N45. d — Focused ultrasound for trauma

4.N46. c — Skull
CT after resuscitation.

4.N47. c — 1500 cc

4.N48. b — The OR

4.N49. d — Discharged
Non-surgical high-risk injuries → admission.

4.N50. b — False
Intervention should NOT be delayed.